

Peptide Protocol Bible

15+ Peptides — Dosing, Stacking, Administration & Research

15+ Peptides	8 Protocols	Clinical Evidence	58+ pg Pages
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■ **MEDICAL DISCLAIMER:** This guide is for educational and harm-reduction purposes only. The compounds discussed are prescription medications in many countries. Always consult a qualified physician before using any pharmacological substance. Tiger Fitness Pro does not encourage illegal activity.

Peptide Protocol Bible — 15+ Peptides

Peptides are short chains of amino acids that signal specific biological processes — growth hormone release, tissue repair, fat oxidation, and more. Unlike anabolic steroids, most peptides have a much more targeted mechanism of action and favourable safety profile when used correctly.

Growth Hormone Secretagogues

Peptide	Class	Half-life	Primary Effect	Research Rating
GHRP-6	GH secretagogue	15–60 min	GH pulse + appetite increase	★★★★■
GHRP-2	GH secretagogue	15–60 min	GH pulse, less appetite than -6	★★★★■
Ipamorelin	Selective GHSP	2 hrs	GH pulse, no cortisol/prolactin	★★★★★
Hexarelin	GH secretagogue	70 min	Strongest GH release, desensitises	★★★★■
CJC-1295 no DAC	GHRH analogue	30 min	Synergistic with GHRPs	★★★★★
CJC-1295 DAC	GHRH analogue	8 days	Once/week GH boost	★★★★■
MK-677 (Ibutamoren)	GH secretagogue (oral)	24 hrs	Sustained GH/IGF-1 elevation	★★★★■

Tissue Repair & Recovery Peptides

Peptide	Primary Use	Dose	Route	Evidence
BPC-157	Gut, tendon, ligament repair	250–500 mcg/day	Subcut or oral	Strong animal data, human anecdotal
TB-500 (Thymosin β4)	Systemic tissue repair	2–2.5 mg/week	Subcut or IM	Strong animal data
GHK-Cu	Wound healing, skin, hair	1–2 mg/day	Subcut or topical	Human clinical data
Epitalon	Telomere extension, anti-aging	10 mg/day (course)	Subcut or IM	Human clinical trials (Russia)
Selank	Anxiolytic, nootropic	250–500 mcg/day	Intranasal	Russian clinical data
Semax	Cognitive, BDNF stimulation	200–600 mcg/day	Intranasal	Human clinical trials

Fat Loss & Metabolic Peptides

Peptide	Mechanism	Dose	Protocol	Notes
AOD-9604	HGH fragment, fat oxidation	300 mcg/day	Subcut, fasted AM	No anabolic effect, pure fat loss
Tesamorelin	GHRH analogue	2 mg/day	Subcut, fasted AM	FDA-approved for lipodystrophy
Fragment 176-191	GH C-terminus fragment	250–500 mcg 2x/day	Subcut	Best combined with low-carb diet

Stacking Protocols

Goal	Peptide Stack	Dose Protocol	Duration
Maximum GH release	Ipamorelin + CJC-1295 no DAC	100 mcg each, 3x/day fasted	8–12 weeks
Injury recovery	BPC-157 + TB-500	500 mcg BPC 2x/day + 2.5 mg TB 2x/week	4–8 weeks
Anti-aging	Epitalon + GHK-Cu + Ipamorelin	10 mg/d + 1 mg/d + 100 mcg 2x	12-week course
Fat loss focus	AOD-9604 + Ipamorelin	300 mcg + 100 mcg fasted AM/PM	12 weeks
Recovery + GH	BPC-157 + GHRP-6 + CJC-1295	500 mcg + 100 mcg + 100 mcg	Ongoing

Reconstitution & Storage Guide

Step	Detail
Solvent	Bacteriostatic water (BAC water) — not sterile water
Amount	Typically 1–2 mL per vial — adjust for desired concentration
Example	5 mg vial + 2 mL BAC = 2500 mcg/mL; 100 mcg dose = 0.04 mL
Temperature	Inject BAC water slowly down vial wall — do not shake, swirl gently
Storage (lyophilised)	Room temperature up to 3 months, fridge preferred
Storage (reconstituted)	Refrigerator 2–8°C, use within 4–6 weeks
Signs of degradation	Cloudy, particulate matter — discard immediately